

**USER INFORMATION
READ CAREFULLY**

**INFUSOL® QSD4
SODIUM CHLORIDE 0.18% w/v & DEXTROSE 4% w/v INTRAVENOUS INFUSION BP**

Composition:

Sodium Chloride BP 1.8 g/L
Dextrose Anhydrous BP 40.0 g/L

Electrolytes:	mmol/L	mEq/L
Sodium Ion (Na ⁺)	30	30
Chloride Ion (Cl ⁻)	30	30
Osmolarity :	283 mOsm/L	
Caloric Value: Approx.	680 kJ/L = 160 kcal/L	

Product Description:

A colourless or faintly strawed coloured solution.
0.18% Sodium Chloride and 4% Dextrose Intravenous Infusion BP represent a slightly hypertonic I.V solution. The outstanding feature of this solution is the markedly low electrolyte content which makes it particularly suitable for use in case of hypertonic dehydration. Since the young are known to require less electrolytes than adults, this solution is ideally suited for use in paediatrics.

Pharmacodynamic:

Glucose with Sodium Chloride supplies energy, along with correction of electrolyte imbalance. It is also supplies water along with essential salts. This is because Sodium Chloride is distributed easily and builds up the salt level rapidly. Thus the second and third step of the dehydration therapy is achieved. This combination of Sodium Chloride with Glucose eliminates the problem of incompatibility which occurs when plain glucose is administered with preserved blood. This results in aggregation of erythrocytes which may lead to transfusion problems.

Pharmacokinetics:

Sodium Chloride is readily absorbed in the body. It is present in all types of body fluids but primarily in extracellular fluid. The osmotic equilibrium is maintained by excretion in urine.
Glucose is rapidly absorbed from the gastrointestinal tract. It is metabolised to carbon dioxide and water with the release of 4 calories/gram of energy. It is readily metabolised. It may deaccelerate body protein and nitrogen losses, promote glycogen deposition and decrease/prevent ketosis in sufficient dose.
Bioavailability : As the solution is directly administered into the blood, it is 100% available.

Indication:

- Hypertonic (esp.hypernatremic) dehydration.
- Supply of free water
- For electrolyte maintenance in paediatrics.
- Caloric supply.
- Vehicle solution for supplementary medication.

Recommended Dosage:

According to individual requirements, approximately 1000mL per day.

Drop Rate: 120 - 180 drops/min corresponding to 360 to 540mL/h

Route of administration: Intravenous (IV)

Contraindications:

- Hyperhydration
- Oedema
- Hypotonic dehydration
- Cardiac insufficiency

Warnings and Precautions:

The compatibility of any additives to this solution should be checked before use. Use with caution in diabetes mellitus. Single dose container. Discard all unused contents. Do not use if leakage is detected. If any visible solid particles, growth or turbidity appears during storage, the product should not be used.

Statement on usage during pregnancy and lactation:

As advised by the physician.

Adverse Effects/Undesirable Effects:

Intravenous Infusions of concentrated dextrose injection are liable to cause local vein irritation. Hypophosphatemia may occur after prolonged use.

Overdose and Treatment:

Dextrose solutions I.V can cause fluid overload resulting in dilution of serum electrolyte concentration, congested state or pulmonary oedema. Significant hyperglycemia, hyperosmolar syndrome and glucosuria may occur with too rapid administration of hypertonic solutions. Hyperglycemia and glucosuria may occur if rate of administration is too fast or in case of metabolic insufficiency. To minimise these conditions, slow infusion rate, monitor blood and urine glucose; if necessary, stop administration of dextrose or/and administer insulin.

Shelf Life:

5 years from manufacturing date in proposed storage condition in LDPE Bottle.
Do not use after expiry.

Storage condition:

Do not store above 30°C.

Dosage form and packaging available:

500mL x 20 LDPE plastic bottle per carton

Manufacturer / Product Registration Holder:

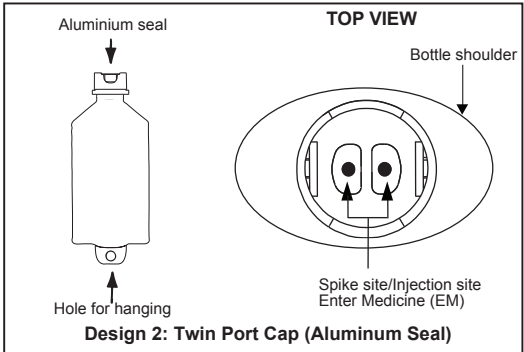
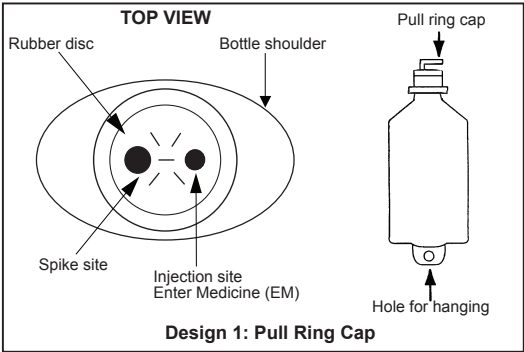


AIN MEDICARE SDN.BHD.

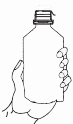
Lot 4933, 4934 & PT2464, Jalan 6/44, Kaw. Perindustrian Pengkalan Chepa 2, 16100 Kota Bharu, Kelantan, MALAYSIA.

HANDLING INSTRUCTION

AIN MEDICARE INTRAVENOUS INFUSION FLUID IN LOW DENSITY POLYETHYLENE (LDPE) BOTTLE
The bottle is made from injectable grade polyethylene.
PARTS OF THE PLASTIC BOTTLE



Top view after pull ring cap is removed. Please refer whichever applicable design.
INSTRUCTION FOR THE USAGE OF IV ADMINISTRATION SET & ADDITION OF DRUG

 Design 1	1. Before use check for any solid particle, growth, turbidity or leakage. Discard the product if particle, growth, turbidity or leakage is found.	 Design 2
 Design 1	2. Design 1: Pull the cap's ring until completely removed. Design 2: Pull the aluminum seal until completely removed by centre line.	 Design 2
 Design 1	3. Additives may be injected through injection site (Refer to Design 1 or 2 whichever applicable) on the rubber. Mix the solution thoroughly.	 Design 2
 Design 1	4. Hold bottle down firmly and insert a spike of administration set into the spike site for Design 1 or for Design 2, pull the other aluminium seal until completely removed before inserting a spike of administration set. <i>(Spike site is meant for spiking of the administration set and is only for single use)</i>	 Design 2